
The Walthamstow Murder

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had revealed themselves in his obstinate disbelief in vaccination, were doubtless brought into undue activity by the business troubles which seem to have recently weighed upon the unfortunate man, with the tragic ending recorded in Monday's daily papers.

GREEN OYSTERS.

In the Biological Section of the recent meeting of the British Association for the Advancement of Science, at Southport, Professor Lankester, F.R.S., gave an account of an investigation he had made into the colouring matter of the green oyster. The cause of the green colour in the oyster was decided fifty years ago by a French oyster-cultivator, who was in the habit of producing them for the Paris market. This gentleman found that in the ponds in Normandy in which these oysters were produced, there was always a green deposit. He thought it was a vegetable substance, and called it *Vibrio ostrearis*. It had since been maintained that it resulted from the contact of the oyster with copper, and it had been alleged that cases of poisoning had in consequence occurred. The explanation, though plausible, as there were minute quantities of copper in the tissues of the oyster, was not the real one. The green substance to which he had referred had been found to be living protoplasm. It came from a microscopical organisation known as the *Mavicula ostrearis*. This organism was swallowed with the water by the oyster, was absorbed into the blood, and the colouring matter was deposited on the surface of the gills and the labial tentacles. It imparted no particular flavour to the oyster, for he would defy any man to distinguish between the green oyster and the white one with his eyes shut.

REGULATION OF MIDWIVES.

IN commenting on the very unsatisfactory position which Sunderland occupies in respect to uncertified deaths, the average being 5.5 per cent. of the total mortality, against that of 2.5 in the twenty-eight large towns, Mr. Harris observes in a recent report that many of these deaths, which occur principally among a colliery population, were newly-born children, who were brought into the world by unskilled and incompetent midwives. Poverty, to some extent, is responsible for this; for it is not always possible for the poorer classes to fee a medical man, and also to pay a nurse, who is required for some time after the infant's birth, not only to look after the mother and child, but also to assist in the house. For these reasons she is selected in preference to the doctor, and, in her ignorance, a frail life is now and again allowed to slip away before any danger is suspected. In these circumstances, Mr. Harris suggests that the medical men of the borough should elect from among themselves an examining board, who should grant certificates of proficiency to all midwives who are willing to present themselves for examination. This would afford protection to a limited extent, as it would enable the poorer classes to judge between the good and the bad. Mr. Harris is also strongly of opinion that legislative measures are necessary to protect the poor against incompetent women—measures which the recent occurrences at Sheffield and Kensington (see BRITISH MEDICAL JOURNAL, vol. i, 1883, pp. 128 and 875) fatally demonstrated to be the only remedy.

SANITARY ARRANGEMENTS AT SMETHWICK.

SMETHWICK is a manufacturing town in South Staffordshire, with a population of upwards of 25,000 persons. It is high time the local sanitary authority bestirred itself. The drainage of the town, or what passes for drainage, is of the most primitive and pernicious description. We are informed that a foul ditch runs through some of the streets, and that an open sewer is to be found in the centre of the town running alongside the principal thoroughfare, Rolfe Street, where it receives the contents of several pigstyes. Another open sewer runs alongside High Park Road, and there receives the

drainage from what is called the "tip." These three sewers all join together at a place which is cheerfully known as "Merry Hell," and which, for many years, has been well-known as a fever-den. At Smethwick the "midden system" is the method generally in vogue for the disposal of excreta; and all the dangers of surface-drainage find abundant illustrations. We hope it may not be unprofitable to call attention to these disgraceful particulars. We have reason to believe that Smethwick is not much worse off in sanitary matters than some of its neighbours amongst the Black Country towns. It is not improbable that the circumstances we point out may have an intimate causal connection with the recent exceptional prevalence of zymotic diseases in the district.

INTERNATIONAL ASSOCIATION OF SURGICAL INSTRUMENT MAKERS.

WE are informed that an international meeting of orthopædic and surgical instrument makers was held at Frankfort-on-Maine on the 10th, 11th, and 12th instant, with the object of constituting a permanent association. This important and difficult trade has made great progress and attained considerable development during the last hundred and fifty years, in harmony with the strides of medical and surgical science, the increasing ingenuity of inventors, and the great demand for instruments of precision and delicacy both for medical and surgical purposes. It was alleged at this meeting, and as one of the reasons for the existence of the association which has been founded, that there is, in these days, a tendency to the wholesale production of imperfect imitations of recognised models, and that this association is founded to protect them and the medical and surgical world against debased reproductions of trustworthy instruments, as well as for the intercommunication of ideas, comparison of new models of inventions, and the general advancement and protection of the interests of the art of surgical instrument making. One hundred and twenty persons, from various parts of the world, have joined the association, which has received the approval of professors of some of the principal medical schools. The executive committee consists of M. Leiter (Vienna), M. Windler (Berlin), M. Walter Biondetti (Basel), M. Krohne (Krohne and Sesemann, London), M. Simsky (Koenigsberg), M. Wendschich (Dresden), M. Laibach (Liège), and M. Fischer (Freiburg in Baden).

THE WALTHAMSTOW MURDER.

WE are informed that the Home Secretary intends to commute the sentence on the convict Gouldstone to penal servitude for life, the evidence of insanity being sufficient to justify a modification of his punishment. It is satisfactory to find that the Home Secretary has promptly yielded to the representations made to him, and ordered an inquiry into the mental condition of the convict. The names of the physicians to whom the inquiry has been entrusted have not been announced, but the probability is that Dr. Orange, of Broadmoor, and Dr. Gower, Her Majesty's Inspector of Prisons, who are generally employed by the Home Office in such investigations, have been instructed to examine Gouldstone. It would be well that the terms of the reference to them should be made public, for no investigation limited to the convict's present mental state, and that did not include his mental state at the time when he committed the crime for which he has been condemned to death, could be regarded as in the least degree satisfactory. Evidence in support of the view that Gouldstone was insane when he destroyed his children has been accumulating rapidly since we commented on the case; and no medical man can read the touching narrative of his wife, and the clear statement of the vicar of the parish in which he lived, without finding in them strong corroboration of the only theory that is consistent with the facts of the case as given in evidence—the theory that the man's arm, when lifted against the children who were so dear to him, was moved, not by criminal design or passion, but by the dire compulsion of disease. The "irrespon-

sible frivolity" and obstinate ignorance of journalism have, in one or two instances, led to sanguinary demands for "the penalty and forfeit" of the law; but, as a rule, the lay and even legal press have recognised the true significance of symptoms in this case; while the organs of medical opinion have been unanimous in their demand for a diagnosis by competent authorities.

LADIES' DRESSES.

DR. HORACE DOBELL, in a letter published in the *Times* of September 18th, draws attention to the bad effects of the "braces system of suspension" for women's dress. He urges that the hips, and not the shoulders, are the best part of the human frame for the support of weight. He adds that "braces restrict the elevation and antero-posterior expansion of the chest; and important as these movements are to men, they are far more so to women." There is doubtless much truth in Dr. Dobell's remarks, but we trust that his assertion that the system of suspension of the clothes from the shoulders is the outcome of the arguments used by "dress reformers" against tight lacing, will not be seized by the ladies as a statement in favour of continuing the use of stays. The evils referred to by Dr. Dobell, doubtless are often produced by the use of braces, and especially when a heavy weight is suspended. This weight is often unnecessarily great in consequence of the looseness of ladies' clothing. If, instead of hanging a quantity of loose clothes round the legs, the garments covering these extremities were chiefly close-fitting, the weight of the clothing might be very much lessened, because when close to the body they preserve the warmth much better than when surrounding the legs loosely. The principle of dual garments might be carried on without altering the external appearance of female dress; for, with the use of warm, close-fitting drawers or trousers, the outer skirts might be made in the usual manner, but of much less thickness and weight. Many ladies now wear neither stays nor suspenders, and yet experience no difficulty in supporting their skirts, chiefly upon the hips.

ANKYLOSTOMUM IN GERMANY.

DR. MENCHE of Bonn describes, in a recent number of the *Zeitschrift für Klinische Medizin*, a case of anæmia from the presence of ankylostomum in the intestine, the first that has been recorded as having occurred in Germany. Hitherto it had been confined to Italy, Egypt, and Brazil, until the epidemic among the workmen in the St. Gothard tunnel showed that the parasite had been imported into Swiss territory. This epidemic is fully described in Dr. Bugnion's contribution to the *JOURNAL* in March 1881. Dr. Menche's patient was a brickmaker, who had never travelled beyond the neighbourhood of Bonn, where he carried on his business. He became very anæmic, and presented all the other symptoms of ankylostomum disease, as described by Dr. Bugnion. After the administration of ethereal extract of male fern, masses of the parasite were voided from the patient's bowels, and the anæmia soon disappeared. Dr. Menche observes that, just as tropical chlorosis and the St. Gothard tunnel epidemic of anæmia have been found to be due to the presence of masses of the blood-sucking ankylostomum in the duodenum, so, as in his own patient, the anæmia common among brickmakers and tileburners may be, generally at least, due to the same parasite.

REMOVAL OF WORKHOUSE PATIENTS.

THE details have lately been published of an inquest recently held on the body of one James Bolton, who died in the Highgate Sick Asylum Hospital a short time after admission. It would appear that Bolton had been an inmate of St. Pancras workhouse, suffering from asthma and bronchitis for some years. Latterly he became so ill that his friends were permitted to see him at all hours. Why this man was sent away is partly explained in Dr. Dunlop's evidence,

who stated that he had seen the patient, and had certified that it was a case for removal, and had therefore sent him away, giving as an additional reason that the workhouse contained about four hundred inmates more than it was certified to hold. He, however, admitted (very unfortunately) that he did not specially examine the man. The *post mortem* examination revealed recent pneumonia, as well as old chest-disease. Medical officers of districts and workhouses cannot be too careful, if they wish to avoid such inquiries, and notably when they are accompanied by special verdicts such as we subjoin: "That the deceased was found dying, and did die in Highgate Infirmary, from the mortal effects of pneumonia and pleurisy following old-standing bronchitis and asthma; and the jurors are further of opinion that the death was accelerated by the removal from the workhouse to the infirmary, and they would call the attention of the medical officer of the workhouse to the urgent necessity for making a special examination of sick patients before their removal, in order that their exact condition might be ascertained in view of the journey they must take to reach the infirmary. We commend this case to the consideration of all medical officers directing such removals.

HOSPITAL ACCOMMODATION FOR INFECTIOUS DISEASES IN LIVERPOOL.

DR. J. STOPFORD TAYLOR, medical officer of health, Liverpool, writes to complain of alleged inaccuracy in the facts and arguments set forth under this heading in the article which we published on the subject last week. He states as an illustration, that "the letter of Mr. David Wainwright has been used on previous occasions, and a full explanation of my conduct in the matter was given, to the satisfaction of the Health Committee, but my critic now supplements it by the following observations:—

"The house in question, 89, Warwick Street, is an eight-roomed house, fronting on a street between forty and fifty feet wide, and by its side-wall forming part of another narrower street which runs at right angles to the first. It is, moreover, tenanted by a highly respectable family, whose last thought would have been to apply for parochial assistance in times of sickness. There was no lack of air-space, either within or without; and the assumption formed, without inquiry, that there was not sufficient lodging and accommodation within it for those who were ill, backed up by the highly practical proceeding of the relieving officer, in the shape of a visit for the purpose of removing them to the workhouse hospital, caused no little astonishment."

"The facts of the case are these: 89, Warwick Street is a sublet lodging-house, and at the time of my visit was kept by a widow, and occupied by three families and two single men. Mr. Wainwright had only recently taken two rooms in the house when his children were attacked with scarlatina, and this gentleman, who had such an horror of an hospital supported by public rates, had no objection to receive the gratuitous assistance of an institution supported by the voluntary contributions of the charitable, so he sent for the dispensary doctor to attend his family. Information of the sickness was given me by the medical attendant, and having no hospital under my control, I sent to the relieving officer for an order for the admission of the patients into the workhouse hospital, which was refused. The relieving officer appeared to have changed his views respecting the house and its tenants since the previous year, when he sent the following letter:—

"I have to report a case of fever at 89, Warwick Street; this is the fourth case of this disease in this house in about a month, one case resulting fatally, therefore I should say that it requires more than ordinary attention."

BATHERS' CRAMP.

A FEW weeks ago, we drew attention to the important question of the nature and causation of the dangerous affection known as bathers' or swimmers' cramp. With the object of suggesting efficient measures for its prevention or mitigation, we showed that